



Surgical Pathology Laboratory

al Pathology Report

In Lab Date:

Report Date:

Submitted by:

Criteria:	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Notes		
Case Is (Circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials		
Date Reviewed	7/17/12	

Telephone:

Location:

Copy To:

CLINICAL INFORMATION:

CA bladder.

Specimen submitted: A. Bladder, prostate and seminal vesicle; B. Left pelvic lymph node; C. Left para aortic tissue; D. Right pelvic lymph node; E. Left distal ureter; F. Right distal ureter

DIAGNOSIS:

A. Bladder, prostate gland, and seminal vesicles (radical cystoprostatectomy):

Bladder:

Invasive high grade urothelial carcinoma.

Tumor grossly and microscopically invades through the muscularis propria into perivesical fat (pT3b). Invasive carcinoma is centered primarily in the trigone and left lateral wall; however, tumor also involves the right lateral wall, bilateral ureteral orifices, and extends into the anterior and posterior walls.

Foci of urothelial carcinoma *in situ* are present throughout the bladder.

Extensive vascular invasion is present, and metastatic tumor deposits are noted in perivesical soft tissue, as well as within prostatic stroma (e.g., slides A46, A48; pM1).

Perineural invasion is present.

Tumor very focally abuts the inked anterior and posterior soft tissue resection margins.

Distal urethral and bilateral ureteral margins are negative for carcinoma.

Prostate:

Multifocal prostatic adenocarcinoma, Gleason score 3+3=6.

Tumors are organ-confined and present as two microscopic foci, present in the right anterior para-apex (slide A38) and right anterior para-base (slide A50).

No angiolymphatic invasion by prostatic carcinoma is identified.

Seminal vesicles and all surgical margins are negative for prostatic carcinoma.

TNM (bladder): pT3b N3 M1

100-03

carcinoma, urothelial, NOS 8120/3

TNM (prostate): pT2a N0 Mx Site: Path: bladder, NOS 607.9

CQCF: bladder, trigone 607.0 7-23-12 eo

Comment: Given the extent of lymph node dissection (see Parts B, C, and D), it is assumed that some of the nodes represent common iliac lymph nodes, which are considered secondary sites of regional lymph node drainage and staged as pN3. Please correlate with clinical and intraoperative findings to confirm the above pathologic "N" stage.

B. Lymph nodes, left pelvis (excision):

Metastatic urothelial carcinoma involving all fourteen lymph nodes (14/14).
The largest tumor deposit measures at least 2.1 cm (microscopic).
Extranodal extension of the carcinoma is identified.

C. Para-aortic tissue, left (excision):

Metastatic urothelial carcinoma involving all five lymph nodes (5/5).
Tumor grossly spans the entire length of the specimen, which measured 7 cm in greatest extent.
Extranodal extension of the carcinoma is identified.

D. Lymph nodes, right pelvis (excision):

Metastatic urothelial carcinoma involving all fifteen lymph nodes (15/15).
The largest tumor deposit measures at least 1.9 cm (microscopic).
Extranodal extension of the carcinoma is identified.

E. Ureter, left distal (excision):

Segment of ureter with acute and chronic inflammation and reactive epithelial changes.
No carcinoma identified.

F. Ureter, right distal (excision):

Segment of ureter with acute and chronic inflammation and reactive epithelial changes.
No carcinoma identified.

GROSS DESCRIPTION:

A. Received fresh, the specimen is labeled "bladder, prostate and seminal vesicles" and consists of bladder, prostate and seminal vesicles. The specimen measures 16 x 12 x 5.5 cm. The bladder measures 7.5 x 7.0 x 5.0 cm. The anterior aspect of the bladder is inked green and the posterior aspect of the bladder is inked black. The right ureter is 0.8 cm in length and 0.9 cm in diameter with a stent in place. The left ureter is 1.4 cm in length and 0.9 cm in diameter also with a stent in place. The specimen is opened from the anterior aspect of the prostate and bladder to reveal a 2.2 x 2.0 x 0.4 cm area of firmness with ulceration and necrosis in the area of the bladder neck/trigone extending into the left and right lateral walls. A section of the ulcerated area is submitted for research. The posterior wall mucosa adjacent to the ulcerated areas is polypoid and erythematous. The specimen is representatively submitted. The prostatectomy specimen includes a 4.5 x 4.5 x 3.0 cm prostate, a 4.5 x 1.7 x 1.0 cm left seminal vesicle, a 7.0 x 0.7 cm left vas deferens, a 4.0 x 2.0 x 1.0 cm right seminal vesicle and a 9.0 x 0.3 cm right vas deferens. The right side of the specimen is inked green, and the left side is inked black. The seminal vesicles and apical margins are shaved and submitted. The apical margin is shaved and submitted. The prostate is serially sectioned from the apex to base with sections designated from A to E, respectively. On sectioning, multiple nodular and cystic areas are seen. The prostate is submitted entirely. Summary of sections: A1-urethral margin; A2-right ureteral margin A3-left ureteral margin; A4-A7 representative bladder neck with ulcer (adjacent to the sections taken for research) ; A8-left UVJ area; A9-right UVJ area with ulceration; A10-prostatic urethra to bladder neck; A11-A13-anterior wall; A14-A16-bladder neck with ulceration; A17-A18-posterior wall; A20-A22-left wall; A23-A25-right wall; A26-A28-dome; A29-A-31-trigone; A32-RSV; A33-LSV; A34-RA apex; A35-RP apex; A36-LA apex; A37-LP apex; A38-RA A; A39-RP A; A40-LA A; A41-LP A; A42-RA B; A43-RP B; A44-LA B; A45-LP B; A46-RA C; A47-RP C; A48-LA C; A49-RL C; A50-RA D; A51-RP D; A52-LA D; A53-LP D; A54-RA E; A55-RP E; A56-LA E; A57-LP E. (Note: R=Right; L=Left; A=Anterior; P=posterior)

B. Received fresh, the specimen is labeled "left pelvic lymph node" and consists of aggregate of red-tan soft tissue measuring 6 x 6 x 2 cm. The lymph nodes are entirely submitted. Summary of sections: B1-one lymph node; B2-B-one lymph node bisected; B4-two lymph nodes; B5-five lymph nodes; B6-B9-one lymph node bisected each cassette.

C. Received fresh, the specimen is labeled "right paraaortic tissue" and consists of a 7 x 2 x 2 cm piece of firm fibrofatty tissue with multiple matted lymph nodes. The specimen is sequentially sectioned and entirely submitted. Summary of sections: C1-C10.

D. Received fresh, the specimen is labeled "right pelvic lymph node" and consists of a 4 x 1 x 1 cm aggregate of fat and lymph node. The lymph nodes are entirely submitted. Summary of sections: D1-one bisected lymph node; D2-D5-multiple intact lymph nodes.

E. Received fresh, the specimen is labeled "left distal ureter" and consists of segment of ureter measuring 1 cm in length and 0.3 cm in diameter. There is stitch marking on one end. The specimen is entirely submitted. Summary of sections: E1-with stitch; E2.

F. Received fresh, the specimen is labeled "right distal ureter" and consists of a segment of ureter measuring 4 cm in length and 0.8 cm in diameter. There is stitch marking on one end. The specimen is entirely submitted. Summary of sections: F1-F5-right distal ureter (F1-with stitch).

***** Electronically Signed Out *****

Attending Pathologist

The attending pathologist whose signature appears on this report has reviewed the diagnostic slides, and has edited the gross and microscopic portions of the report in rendering the final diagnosis.

Slides: A-57, B-9, C-10, D-5, E-2, F-5